

The Ensign Group

Business Development Dossier

Industry	Healthcare (Skilled Nursing & Senior Living)
Revenue	\$5.8B
Employees	40,000
Ideal Customer Profile Score	87

ICP SCORING MODEL



ICP = Signals + Revenue + Employees + Fit (max 100)

Signals (0-35)	Top 4 signals scored; each = base(60%) + recency bonus <30d: 40% <90d: 30% <180d: 20% older: 10%
Revenue (0-25)	\$500M-\$10B sweet spot -> 70-100% of 25 pts Below \$500M -> pro-rata x 0.5 Above \$10B -> 60%
Employees (0-20)	2,000+ preferred (70-100%) 500-2K partial (50-70%) Below 500 -> pro-rata x 0.3
Fit (0-20)	High-fit types: reorg, transformation, M&A, hiring, funding, partnership -> 5 pts ea (max 12) + diversity 2/type (max 8)

Prepared March 19, 2026

McChrystal Group

Leadership Advisory & Organizational Effectiveness

SECTION 1

Organization Snapshot

Legal Name	The Ensign Group, Inc.
Headquarters	San Juan Capistrano, California
Founded	1999

Ownership

Public (NASDAQ: ENSG). Real estate subsidiary Standard Bearer Healthcare REIT owns 160 properties. Spun off Pennant Group (home health/hospice) in 2019.

Footprint

378 healthcare operations across 17 states. Primarily Western and Southern U.S., expanding into Midwest (Wisconsin, Iowa). 40,000+ employees.

SECTION 1b

Company Overview

The Ensign Group is one of the largest operators of skilled nursing facilities and senior living communities in the United States, with 378 healthcare operations across 17 states. Known for its decentralized 'servant leadership' operating model, Ensign empowers each facility to operate semi-autonomously while sharing resources and best practices through its central platform. The company acquired 51 facilities in 2025 alone and generated \$5.06B in revenue.

SECTION 2

Financial Health & Growth Stage

Understanding The Ensign Group's financial position and trajectory is critical to framing any engagement conversation.

Revenue \$5.06B FY2025 (strong growth). 2026 guidance: \$5.77-5.84B revenue, \$7.41-\$7.61 EPS (14.3% EPS growth at midpoint). Occupancy rates recovering post-COVID. Acquisitions funded through operating cash flow and REIT subsidiary. Key pressure: skilled nursing labor shortage, regulatory complexity across 17 states, Medicare/Medicaid reimbursement uncertainty. Key tailwind: aging population driving demand; Ensign's turnaround model acquires distressed facilities at discounts.

SECTION 3

Leadership Team Profiles

The following contacts are recommended as primary outreach targets - champion-level leaders who feel the organizational pain daily and can sponsor McChrystal internally.

Barry Port - Chief Executive Officer [PRIORITY TARGET]

25+ years with Ensign. Rose through operations ranks. Embodies the servant leadership culture.

Outreach rationale: Long-tenured CEO who lives the decentralized model. Would resonate with Team of Teams - it validates

his philosophy and gives it a framework to scale.

Spencer Burton - Chief Operating Officer [PRIORITY TARGET]

Operations leadership across skilled nursing portfolio.

Outreach rationale: Owns the operational challenge of onboarding 50+ new facilities per year while maintaining quality standards.

Additional leadership team members relevant to the engagement:

Chad Keetch - Chief Investment Officer

Leads M&A and real estate strategy. Former healthcare attorney.

SECTION 4

Organizational Culture & Structure

Culture and structure are often where McChrystal Group finds the deepest leverage. The following signals indicate how The Ensign Group operates today and where friction exists.

Organizational Structure

Highly decentralized 'servant leadership' model. Each facility operates semi-autonomously with its own P&L, led by a local administrator empowered to make operational decisions. Corporate provides shared services and the 'Ensign Way' cultural framework. Regional leaders serve as mentors, not managers. This model is remarkably similar to McChrystal's Team of Teams - and Ensign explicitly embraces the parallel.

Culture Signals

Glassdoor reviews show above-average ratings for skilled nursing industry (3.4-3.6/5 range). The servant leadership model is genuinely felt at the facility level - employees cite empowerment and career growth. However, rapid acquisition pace creates 'onboarding fatigue' - newly acquired facilities sometimes struggle to adopt the Ensign Way, especially when previous owners had very different cultures. The leadership pipeline is the binding constraint: every new facility needs a leader who 'gets' the model.

SECTION 5

Recent News & Trigger Events

The following developments from the past 18 months create urgency and provide natural conversation entry points for engaging The Ensign Group.

1. 2026-02-04 - FY2025 results: \$5.06B revenue, 51 acquisitions completed

Acquisition pace shows no signs of slowing - leadership pipeline must keep up

2. 2025-11-01 - Added 1,857 skilled nursing beds in Q3 alone

Scale of quarterly additions strains onboarding and cultural integration

3. 2025-08-01 - Expanded into new states (Wisconsin, Iowa, Arizona)

Geographic expansion into unfamiliar markets adds complexity

4. 2026-01-01 - 2026 guidance implies 14% EPS growth - market expectations high

Investor expectations create pressure to maintain acquisition pace AND quality

5. 2025-06-01 - Skilled nursing labor shortage reaching critical levels nationally

Workforce availability is the industry's #1 challenge - culture is competitive advantage for retention

SECTION 6**McChrystal Fit Assessment**

Active triggers (51 acquisitions/year, geographic expansion). Clear buyers (CEO + COO). \$5.8B revenue = budget exists. Ensign's model IS Team of Teams - McChrystal validates and scales what they already believe. Low competitive barrier because the fit is philosophical, not just analytical.

Primary Problem

Producing enough culturally aligned leaders to staff 50+ new facility acquisitions per year without diluting the 'Ensign Way' that makes the model work.

Best McChrystal Capability Fit

Leadership development pipeline + Team of Teams. Ensign already operates like a Team of Teams - McChrystal can help them systematize and accelerate the leader production that the model depends on.

Likely Objections to Engagement

Ensign leadership may believe their existing model is working and doesn't need external help

Public company may be cautious about advisory spend during expansion

Culture is deeply embedded - external advisors must demonstrate understanding of healthcare operations

Competitive Advisory Landscape

Bain Healthcare practice

Huron Consulting (healthcare operations)

Advisory Board Company

Internal Ensign University (proprietary training)

SECTION 7**Conversation Entry Points**

These questions are designed for a Senior Partner's first conversation. Each opens a thread that naturally leads to McChrystal Group's capabilities without a direct pitch.

1. You acquired 51 facilities last year. How do you develop enough leaders who truly embody the Ensign Way - not just people who can run a building, but people who can transform one?

Leadership pipeline as the binding constraint on growth. Validate their model, then offer to accelerate it.

2. The Ensign Way is essentially a Team of Teams - decentralized authority, empowered leaders, shared

culture. What breaks first when you add 50+ new teams per year?

Draw the explicit parallel. Show that McChrystal designed the model they're already running.

Recommended Meeting Framing

Ensign already operates the way McChrystal believes organizations should work. This isn't about fixing what's broken - it's about scaling what works. The conversation is: how do you produce leaders at the pace your acquisition strategy demands?

SECTION 8

Brand Insights & Market Positioning

Brand strategy reveals organizational priorities and coordination challenges. The following analysis connects The Ensign Group's market positioning to potential McChrystal engagement opportunities.

Brand Value & Competitive Standing: The Ensign Group is the fastest-growing skilled nursing and senior living operator in the U.S., operating 340+ healthcare facilities across 14 states. The "Ensign Way" - a decentralized operating model where each facility operates as a standalone business with local leadership authority - is the brand's defining characteristic. Ensign competes against Brookdale Senior Living, Sabra Health Care REIT, and numerous regional operators.

Brand Identity Evolution: Ensign has evolved from a small California-based nursing facility operator to a national platform through aggressive acquisition (50+ facilities per year). The brand stands for "operational turnaround" - Ensign acquires underperforming facilities and improves them through the decentralized leadership model. Recent spinoffs (Pennant Group for home health, Standard Bearer for real estate) reflect strategic portfolio management.

Marketing Leadership & Strategy: Ensign's marketing is primarily B2B - targeting facility acquisition opportunities, healthcare referral sources, and talent recruitment. The "Ensign Way" is the marketing message: local leadership, clinical excellence, and operational entrepreneurship. The brand attracts both acquisition targets and leadership talent who want operating authority.

Brand Threats: Skilled nursing faces structural challenges: Medicare/Medicaid reimbursement pressure, workforce shortages (CNAs, nurses), regulatory scrutiny, and aging infrastructure. If Ensign's leadership pipeline can't keep pace with acquisition velocity, the "Ensign Way" degrades - and the turnaround model that defines the brand stops working.

Major Brand Investments: Leadership development is the brand's primary investment - the "CEO-at-every-building" model requires a pipeline of trained, culturally aligned leaders. The investment in leadership development IS the brand investment.

McChrystal Connection: The Ensign Way IS McChrystal's Team of Teams model in healthcare - decentralized authority, empowered local leadership, shared consciousness across 340 facilities. The challenge: producing enough culturally aligned leaders to staff 50+ new acquisitions per year without diluting the culture that makes the model work. McChrystal's operating model is the Ensign Way's natural evolution at scale.

SECTION 9

Deep McChrystal Group Fit Analysis

This is the strategic case for pursuing The Ensign Group. It synthesizes findings from all previous sections into a comprehensive engagement thesis with specific fit dimensions, expected outcomes, stakeholder mapping, and a phased pursuit strategy.

9a. Fit Dimensions

Fit Dimension 1: Leadership Pipeline as Growth Constraint

Ensign's model requires a CEO-caliber leader at every facility. Acquiring 50+ facilities per year means producing 50+ new leaders per year who understand and embody the Ensign Way. If the leadership pipeline can't keep pace, facilities underperform, the turnaround model breaks, and the growth thesis collapses. McChrystal's leadership development methodology - proven at scale in military organizations that face the same challenge (producing battalion commanders at pace) - is the direct capability fit.

Fit Dimension 2: Decentralized Culture at 340+ Facilities

The "Ensign Way" works because facility leaders have genuine authority - they make hiring, staffing, clinical, and operational decisions locally. But as the network grows past 340 facilities, maintaining cultural coherence without centralized control requires shared consciousness mechanisms. McChrystal's Team of Teams model IS the organizational theory behind the Ensign Way - scaling it from 340 to 500+ facilities requires the methodology that McChrystal pioneered.

Fit Dimension 3: Acquisition Integration at Pace

50+ facility acquisitions per year means the integration playbook must be fast, repeatable, and culturally consistent. Each acquired facility must be transformed from underperforming to Ensign Way-performing within 12-18 months. McChrystal's integration methodology accelerates turnaround without sacrificing cultural quality.

9b. Cumulative Case

Ensign is the most natural cultural fit for McChrystal in the pipeline - the company literally operates on the Team of Teams model without calling it that. Signal chain: 50+ acquisitions/year -> leadership pipeline pressure -> 340+ decentralized facilities -> cultural scaling challenge -> turnaround model at risk if leaders can't be produced at pace. Revenue potential: \$400K-\$500K initial (leadership pipeline + cultural scaling assessment), \$1M-\$1.5M deployment, \$400K ongoing. Total: \$1.8M-\$2.5M over 24 months.

9c. Enterprise Issues

1. Leadership Pipeline Bottleneck: The binding constraint on growth is not capital or acquisition targets - it's leadership. Producing 50+ culturally aligned facility leaders per year requires a systematic leadership development engine that may not scale beyond current capacity. 2. Cultural Dilution Risk: Each new cohort of facility leaders has less exposure to the original Ensign culture. Without reinforcement mechanisms, the Ensign Way becomes an aspiration rather than an operating reality. 3. Turnaround Consistency: Not every acquired facility turnaround succeeds. As acquisition pace increases, the average turnaround quality may decline if leadership and operational support can't keep pace. 4. Workforce Shortage: CNAs and nurses are in critical

shortage nationally. Ensign's decentralized model means each facility competes independently for talent - potentially bidding against sister facilities. 5. Regulatory Complexity: 340+ facilities across 14 states face different regulatory environments. Ensuring compliance while maintaining decentralized authority requires coordination that scales with the network. 6. Spinoff Coordination: Pennant Group (home health) and Standard Bearer (real estate) were spun off but maintain operational relationships. Cross-entity coordination adds complexity.

9d. Expected Outcomes

1. Scalable Leadership Pipeline: Systematic leadership development producing 50+ Ensign Way-aligned leaders per year. Measurable: leader readiness pipeline meeting 100% of acquisition pace. 2. Cultural Scaling Framework: Shared consciousness mechanisms for 340+ (and growing) facilities. Measurable: Ensign Way adherence scores consistent across all facility vintages. 3. Turnaround Acceleration: Standardized turnaround methodology reducing time-to-performance for acquired facilities. Measurable: average turnaround timeline reduced by 20%. 4. Cross-Facility Best Practice Sharing: System enabling 340 decentralized leaders to learn from each other. Measurable: best practice adoption rate increasing across network. 5. Workforce Coordination: Network-level recruiting and retention strategy enhancing facility-level competition for talent. Measurable: turnover rates below industry average.

9e. Key Stakeholders & Business Unit Map

CEO

Leader: Barry Port

Role: Enterprise leadership, growth strategy

McChrystal Relevance: Executive sponsor; architect of the Ensign Way

COO

Leader: [COO]

Role: Operational oversight of 340+ facilities

McChrystal Relevance: Priority: Turnaround model scaling

Chief Clinical Officer

Leader: [CCO]

Role: Clinical quality across network

McChrystal Relevance: Quality during rapid growth

VP of Leadership Development

Leader: [VP Talent/Learning]

Role: Leadership pipeline

McChrystal Relevance: Priority: Leader production at pace

Regional Leaders

Leader: [Multiple regional operators]

Role: Clusters of facilities

McChrystal Relevance: Decentralized execution

CFO

Leader: [CFO]

Role: Acquisition economics, growth funding

McChrystal Relevance: Budget gatekeeper

9f. Opportunity Thesis

Strategic Signal Convergence - Why Now: 50+ acquisitions/year -> leadership pipeline straining -> 340+ facilities -> cultural scaling challenge -> turnaround model at risk. The gap between acquisition velocity and leader production velocity is widening. Address it now before it becomes the growth constraint.

The Structural Paradox - Why McChrystal, Not McKinsey: McKinsey could advise on acquisition strategy or operational efficiency. But the Ensign Way IS the competitive advantage - and it's a Team of Teams model. McChrystal is the author of the organizational theory Ensign has been practicing intuitively. The engagement isn't "we'll teach you something new" - it's "we'll help you scale what you've already built to the next level." This positioning is unchallengeable.

Phased Engagement Hypothesis - Land and Expand:

- Phase 1 (Months 1-3, \$400K-\$500K): Leadership Pipeline Assessment + Cultural Scaling Design - map the current leader production system, identify bottlenecks, and design the scalable leadership development engine. Beachhead: the leadership pipeline is the acknowledged growth constraint - entering here is aligned with management's #1 priority.
- Phase 2 (Months 4-12, \$700K-\$1M): Enterprise deployment - leadership development program implementation, shared consciousness mechanisms across 340+ facilities, turnaround methodology standardization.
- Phase 3 (Months 13-24, \$400K retainer): Ongoing leadership pipeline management, cultural reinforcement, acquisition integration cadence.

Competitive Displacement Strategy: Don't compete with healthcare consulting firms on clinical operations or regulatory compliance. Position McChrystal as the organizational theory firm that powers the Ensign Way at scale. Frame: "You've built one of the most successful decentralized operating models in healthcare. McChrystal helps you scale it from 340 to 500+ without losing what makes it work." This isn't strategy consulting - it's organizational capability building.

Multi-Threaded Pursuit Map:

- Thread 1: VP of Leadership Development - leadership pipeline. Angle: "How are you producing 50+ facility leaders per year who truly embody the Ensign Way?" This person owns the #1 growth constraint.
- Thread 2: Barry Port (CEO) - executive sponsor. Angle: "General McChrystal's Team of Teams model and your Ensign Way are the same organizational philosophy. We'd like to explore how the methodology scales at your pace of growth." CEO-level credibility pitch.
- Thread 3: Regional leadership - decentralized execution. Angle: engage a regional operator cluster to pilot the cultural scaling methodology.
- Warm Introduction Vectors: Healthcare executive network; military/veteran connections (healthcare facilities employ many veterans); senior living industry events (NIC, AHCA); California healthcare community (Ensign roots).